

Referral to Children's Eating Disorder Team- South Staffordshire

We are required to register the full postcode, GP details and NHS number of all referrals. Please include this information in your referral.

(Please consult referral guide for more information regarding suspected Eating Disorders)

Please complete this form and email to camhseatingdisorders@mpft.nhs.uk

! It is important that you fill in all areas of this form to allow appropriate and safe triage and assessment, and to avoid delays in processing. Unfortunately, we may not be able to accept incomplete referrals and this may cause significant delay to the triaging process. **!**

As Eating Disorders are serious mental health problems with significant medical risk, please note that the GP remains responsible for medical care and monitoring of the patient until patient is linked in with this service. Please refer to the Medical Emergencies in Eating Disorders Guidelines (MEED).

FAILURE TO INCLUDE ALL PHYSICAL HEALTH OBSERVATIONS WILL RESULT IN THE FORM BEING RETURNED FOR COMPLETION.

REFERRER DETAILS

Name (Print):

Signature:

Organisation: Darwin Medical Practice
Staffordshire, WS7 0AQ

Address: Greenwood Health Centre, Lichfield Road, Burntwood,

Email: darwinmedical.practice@nhs.net

Telephone: 01543682654

Date of referral	21 May 2026	Priority:	Routine / Urgent
Brief description of reason for referral:			

DEMOGRAPHIC INFORMATION

Child/young person's details and parent/carer details

Full name	MOUSE, Minnie (Miss)	Address	10, That Street, This City, Staffordshire, WS14 8WW
Date of birth	10-Oct-1960		
NHS No.		Telephone no.	01342187332
Gender	Female	Mobile no.	
Ethnicity		Email	
Main spoken language at home		Interpreter required?	Yes ☐ No ☐ Details:
Disabilities Physical/Communication impairments, especially if requires assistance with arranging appointments?	☐ Yes ☐ No Details:		
GP Details (if not referrer)	As above	GP address & Contact number	As above

Name of parents/carers (whom patient lives with)		Relationship to child	
Telephone no.		Email	
Who holds parental responsibility? (parent/carer/Local Authority (LAC) Please give contact details if not already shown above.			
Is the parent aware of the referral being made?			
And has consent to share information been given?			
Name		Telephone no.	
Address		Email	

REASON FOR CURRENT REFERRAL (Continue on separate sheet if required)		
Please indicate what is required from our service and the eating disorder psychopathology present.		
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Please provide details of significant history, concerns and any risks.		
Past Medical History -- Include only relevant medical history:		
Problems:		
Problems		
Active		
Date	Problem	Associated Text
12-Feb-2026	Migraine	
28-Mar-2014	Screening offered	
25-Mar-2014	Screening offered	
10-May-2011	Asthma	
10-May-2011	Depot contraceptive given	
10-May-2011	Intramuscular injection of vitamin B12	- BN12345 02/13
04-May-2011	Chronic obstructive pulmonary disease	
02-May-2011	[D]Musculoskeletal chest pain	
26-Apr-2011	Lower resp tract infection	
26-Apr-2011	Lung cancer	
29-Mar-2011	Acquired hypothyroidism	
29-Mar-2011	Allergic reaction to insect bite	
23-Mar-2011	Coeliac disease NOS	
15-Sep-2010	Deep vein thrombosis	
Significant Past		
Date	Problem	Associated Text
09-Jul-2025	Pregnancy	

24-Aug-2011 Blue baby
 20-Apr-2011 Bowel cancer screening programme
 22-Mar-2011 Essential hypertension
 03-Nov-2010 Sore throat symptom

Summary:

Any other agencies currently involved/involved in the past with the child/young person/family; and any services for which they are on a waiting list, and attach any relevant reports.

Physical health assessment: PLEASE NOTE THAT THESE SHOULD BE DONE WITHIN 2 WEEKS OF THE REFERRAL BEING SENT

Date of physical health assessment		Allergies	Allergies	
			Date	Description Associated Text
			29-Mar-2011	Allergic reaction to insect bite
			05-Mar-2011	Adverse reaction to Garlic
			05-Mar-2009	Adverse reaction to Dusting Powders
Weight (kg)	27-Sep-2017 O/E - weight : 65 kg	Height (metres)	27-Sep-2017 O/E - height : 159 cm	
Weight history – highest & lowest weights + dates	- -	Temperature	No events found.	
Blood Pressure sitting standing	No events found.	Heart Rate sitting standing		
	No events found.			

Date Bloods / ECG requested – Please see guidance sheet for what bloods are needed:
 (If results available please attach)

Risk section (Health Professionals only)

ONE OR MORE TICKED BOXES BELOW INDICATES URGENT ASSESSMENT REQUIRED BY YOUR LOCAL DUTY PAEDIATRIC SERVICE AS EMERGENCY ADMISSION MAY BE REQUIRED

- ☐ Low weight <70% weight for height
- ☐ Rapid weight loss of >1kg/week for 2 consecutive weeks and/or history of rapid significant weight loss
- ☐ Acute food or fluid refusal/intake <500 calories per day
- ☐ Heart rate less than 40bpm (though would also be concerned with HR 40-50bpm with other risk factors,) symptomatic postural tachycardia
- ☐ Abnormal ECG – in particular prolonged QT, arrhythmia associated with malnutrition and/or electrolyte disturbance
- ☐ Blood pressure below the 0.4th centile for age/gender and/or postural drop of more than 20mmHg

☐ Core Temperature < 35.5 degrees ☐ Hypoglycaemia <3mmol/L ☐ Evidence of Purging ☐ Blood results disturbance including electrolyte abnormalities such as hypokalaemia <3mmol/L, hypo or hypernatremia, raised urea/creatinine and raised transaminases ☐ Concerns around risk with self-harm/suicidality	
Has the young person experienced any fainting or dizziness, If yes please provide details i.e when, how often	
Has the young person experienced any chest pain or palpitations?, If yes please provide details i.e when , how often	
Date of Menarche	Current Menstrual History
Has the young person noticed hands/ feet feeling cold/ looking blue -	

Medication			
Current acute medication in the last 1 month			
Name of medication	Dose	Prescribed by / obtained from	Duration
Please see attached			

Eating Behaviours			
Are there any fears relating to food & Eating	☐ YES ☐ NO Please provide details:	Are there any fears around gaining weight	☐ YES ☐ NO Please provide details:
Does the young person count calories	☐ YES ☐ NO Please provide details: Average calories per day _____	Are meal times a source of tension	☐ YES ☐ NO Please provide details:
Fluid intake	☐ Refusing fluids (with reduced urine output) ☐ Restricting fluids ☐ Drinking excessive amount (fluid loading) Please provide details of average drunk in one day _____	Restricting dietary intake	☐ Acute food Refusal (had little or no intake for >2 days) ☐ Choosing low calorie food ☐ Skipping meals (e.g., eating <3 main meals per day) ☐ Eating smaller than average portions ☐ Excluding entire food groups e.g., dairy/carbohydrates/meat/veg/fruit

Food avoidance based on any of the following:	☐ Disturbed body image (e.g., believes they are overweight when they are objectively underweight) ☐ Texture of food ☐ Smell of food ☐ Taste of food ☐ Appearance of food ☐ Lack of hunger/appetite/ interest in eating ☐ To help manage negative emotions. ☐ Fear of consequences of eating e.g. - choking ☐ - vomiting ☐ - gastrointestinal symptoms ☐ - weight gain ☐ - Other ☐ (please specify):
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Other Behaviours

Does young person make themselves sick after eating	☐ YES ☐ Multiple daily episodes ☐ More than 3 times per week ☐ NO	Taking laxatives/ diet pills	☐ Yes ☐ No ☐ Multiple daily episodes ☐ More than 3 times per week
Eat large quantities of food (i.e binge eating)	☐ YES ☐ NO	Eat food/ meals fast or slow	
Have any difficulty concentrating	☐ YES ☐ NO	Activity (please provide details of all activity undertaken including PE, walking to and from school)	☐ Less than 1 hour per day ☐ 1-2 hours per day ☐ More than 2 hours per day
Please provide details of any other concerns moods, anxiety etc)	-		

Further Information

Do the parents/carers (who have parental responsibility) consent to this referral?	☐ YES ☐ NO	If 16 or over, has consent been given by the young person?	
Has the child/ young person/family had previous involvement with this or any other CAMHS?	☐ YES ☐ NO ☐ Unsure	If Yes	Date : Reason:
Has the child/young person ever had a Child Protection Plan?	☐ YES ☐ NO ☐ Unsure	If Yes	Date : Reason:
Is the child/young person/family currently involved in Legal Proceedings relating to the child/young person?	☐ YES ☐ NO	If Yes , please give brief details	
Are you aware of any domestic violence issues in this family?	☐ YES ☐ NO	If yes, Please provide brief details	
Are there any other matters, such as culture, language, illness, religion, or disability that we may need to consider when getting in touch with the family?			

Name (Print)		Signature	21 May 2026	Date	
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Medication

Acute

Drug	Dosage	Quantity	Last Issued On
Denosumab 60mg/1ml solution for injection pre-filled syringes	For Subcutaneous Injection every 6 months in clinic	1 pre-filled disposable injection	23-Jul-2015
Ramipril 2.5mg capsules	One To Be Taken Each Day	28 capsule	
Influenza vaccine (split virion, inactivated) suspension for injection 0.5ml pre-filled syringes (Sanofi Pasteur)	Immediately	1 pre-filled disposable injection	
Hydroxocobalamin 1mg/1ml solution for injection ampoules	1ml	1 ampoule	
Influenza vaccine (split virion, inactivated) suspension for injection 0.5ml pre-filled syringes (Sanofi Pasteur)	Immediately	1 pre-filled disposable injection	

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Co-codamol 8mg/500mg effervescent tablets	One To Be Taken Four Times A Day	2 tablet
Ramipril 1.25mg capsules	One To Be Taken Each Day	28 capsule
Senna 7.5mg tablets	One Or Two To Be Taken Each Day As Directed	30 tablet
Trimethoprim 200mg tablets	One To Be Taken Twice A Day	6 tablet

Repeat

Drug	Dosage	Quantity	Last Issued On
Tramadol 50mg capsules	One Or Two To be Taken Every Four Hours When Necessary. Maximum 8 Tablets in 24 Hours	100 capsule	
Amoxicillin 250mg capsules	One To Be Taken Three Times A Day	15 capsule	
Trimethoprim 200mg tablets	One To Be Taken Twice A Day	14 tablet	

Repeat Dispensing

Drug	Dosage	Quantity	Last Issued On
Irriclen sodium chloride 0.9% irrigation solution aerosol spray (ConvaTec Ltd)	daily	240 ml	

Allergies

Date	Description	Associated Text
29-Mar-2011	Allergic reaction to insect bite	
05-Mar-2011	Adverse reaction to Garlic	
05-Mar-2009	Adverse reaction to Dusting Powders	